

Emergency Management of Hyponatraemia

Take blood samples prior to starting treatment (if possible)

- Urine Osmolality and Urinary sodium
 - Random blood glucose
 - U&Es
 - TSH, cortisol

Mild Hyponatraemia
Na: 130 - 135

Moderate Hyponatraemia
Na: 125-129

Severe Hyponatraemia
Na: <125

Establish Fluid Status

Hypovolaemic

Euvolaemic

Hypervolaemic

Signs of cerebral oedema present?

Nausea, vomiting, low GCS, ataxia, confusion, headache

Yes

No

Goal: increase by 4-6mmol in 1st 2-4 hours

AVOID correction of more than 10mmol/24 hours

For hypovolaemia and euvolaemia use 150ml of 2.7% hypertonic saline via central or large peripheral vein

Increase less than 4mmol at 4 hours

Symptomatic improvement OR
Na increased by more than 5mmol

Repeat 150ml of 2.7% hypertonic saline and **CONSULTANT REVIEW**

Diagnose and manage cause with **CONSULTANT REVIEW**

Euvolaemia
Underlying cause?

SIADH
see separate pathway

Water intoxication

Fluid restriction and CONSULTANT REVIEW

Hypovolaemia
Review factors of cause and PMHx such as burns, GI (diarrhoea, vomiting, dehydration), renal

NaCl 0.9% 1000ml over 6 to 8 hours

Add on [if hypovolaemic]

END OF PATHWAY

Refer to senior, obtain urine dip, and treat underlying cause (liver failure, nephrotic syndrome)

Patients with severe, symptomatic hyponatraemia and at high risk of ODS should have hourly monitoring until Na⁺ increase by 4-6 mmol/l and then 4-6 hourly (using blood gas machine)